



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : كوثر عادل عبدالعزيز عبدالحاميد

العنوان : مم

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 21-09-2026

رقم البطاقة : 28409200108303 رقم التليفون : 01284831607

العمر (تاريخ الميلاد) : 20-09-1984 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: كوتر عادل عبدالعزيز عبد الحميد
National ID: 28409200108303



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Further investigations and imaging

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



كوثر
عادل عبدالعزيز الحميد
٤٠ ش الاحرار - جمال عبدالناصر - شنتيل
مركز اوسيم - الجيزة

٢٨٤٠٩٢٠٠١٠٨٣٠٣
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KJ1747475

٢٠٢٤/٠١ ٢٨٤٠٩٢٠٠١٠٨٣٠٣

ربة منزل

أنثى مسلمة
محمد فتحي خميس
البطاقة سارية حتى ٢٠٢٠/١٢/٣١




MDT FORM

Name:	Kawsar Adel Abdelaziz Abdelhamed	MRN:	2735
Age:	42 Y	ID:	2267-2025 / DMS 2735--28409200108303
Menopausal Sta:	pre	Phone:	01158278703
NRS SCORE:	ps1	Marital Status:	MARRIED
OMI:		Family History:	-VE
CO Morbidity:	M:MTN S;APPENDLECTOMY	Reason Of MDT:	POST MRM

Clinical Data:

BX: IDC G2 triple-ve
ER: 0/8 PR: 0/8 HER2: +1 KI67: 25

R

L

Sonomammography:

Mets Workup:

CT CAP:FREE
BONE SCAN :FREE

History:

prem rt breast cancer ct4b n1 mo brca1,2 no mutation rec nact 3 ac 12 taxol carboplatin last 1/3/2026 with good regressive course
AFTER ASSESSMENT IN SURGERY CLINIC STILL NOTED RESIDUAL CENTRAL AND INNER QUADRANT EDEMA GOOD FLAPS
BY HX GIVEN FROM PT AND AFTER REVIEWING INITIAL SONOMAMMO REPORT PT INITIALLY IDC T4D NOT T4B.
The patient underwent MRM

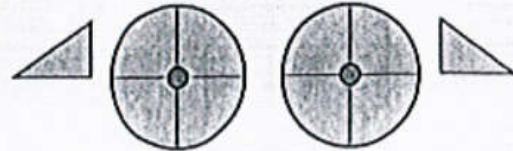
Recommendations:

In view of having TNBC ypT0 N1, consider:

- 1- PORT
- 2- adjuvant capecitabine for 6 months
- 3- IHC for residual disease

MDT Outcome & Plan

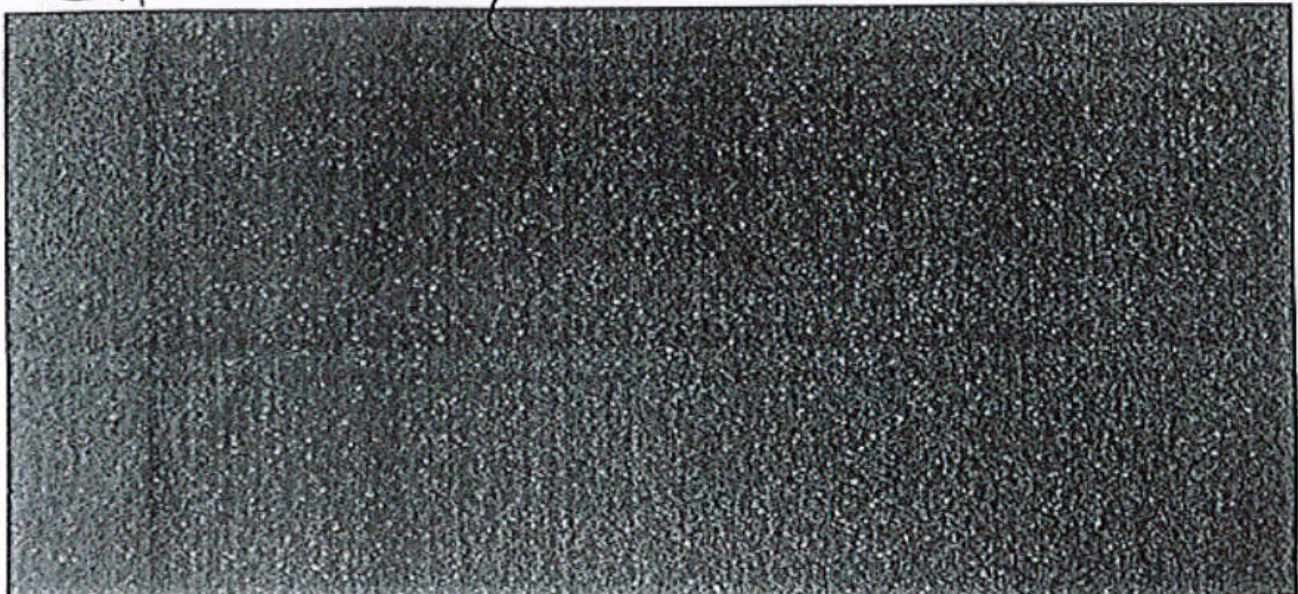
- 1- PORT
- 2- adjuvant capecitabine for 6 months
- 3- IHC for residual disease



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
DE HAYAN EZZAT					



Patient Name : الأستاذ / كوثر عادل عبد العزيز	Request No. : 13003468
Patient No. : 28409200108303	Request Date : 18/11/2025
Path No. : SR-1662/25	Verification Date : 01/12/2025
Sex : Female	Financial Category : جوسداف زومسي - نفقة الدولة
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Right breast mass.

Nature of specimen

Slide revision

Gross examination

Five slides labelled by (318/25) were received .

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Positive , cribriform , solid , comedo , subtypes , low grade
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Absent .
- * Microcalcification : Absent
- * Perineural invasion: Absent
- IPT :
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: No nuclear reaction in tumor cells (Allred score 0/8)
- * HER2: Faint incomplete membrane reaction in >10 % of tumor cells (1+)
- * Ki 67: 25 %

Diagnosis

RIGHT BREAST MASS, SLIDE REVISION

-Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)
(according to Nottingham grading system of breast cancer) , with positive duct carcinoma
insitu , cribriform , solid , comedo sutypes , low grade

Verified by :

Dr.Heba Hasan

Printing Date : 07/12/2025 08:46:55

Page 1 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / كوثر عادل عبد العزيز	Request No. : 13003468
Patient No. : 28409200108303	Request Date : 18/11/2025
Path No. : SR-1662/25	Verification Date : 01/12/2025
Sex : Female	Financial Category : جوسف رومي - نفقة الدولة
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

IPT:

* ER: Negative(score 0/8).

* PR: Negative (score 0/8).

* HER2: score +1

* Ki67: 25 %

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

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PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: كوثر عادل عبدالعزيز	Age: 41years	Sex: female
Request type: مبادره	Sub request type:	
Hospital no:	Pathology No.: 318B-2025	
Department: صحة مراه	Date of Result: 4/8/2025	
Referred by: د/مروه حسن	Receiving date: 22/7/2025	
Nature of specimen: tru cut biopsy	I.D. Number: 28409200108303	

Clinical diagnosis: Right breast mass

Gross pathology :

Four gray white cores up to 1cm long each, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- Ki67

Microscopic picture: Sections examined revealed breast core tissue showed invasive duct carcinoma of no special type (NST). The tumor forms sheets, clusters, trabeculae and tubules of anaplastic ductal cells infiltrating among desmoplastic stroma mild lymphocytic response. Tumor cells showed moderate anaplasia and moderate tendency to form tubules. Also seen DCIS component of intermediate grade, solid subtype.

Immunohistochemical Result:

ER: Moderate nuclear staining in <10% of tumor cells (Allred Score 4/8).

PR: Moderate nuclear staining of 1% of tumor cells (Allred Score 3/8).

Her2: Weak incomplete membranous staining of of tumor cells (Score 1+)

Ki67: Positive nuclear staining of 25% of tumor cells

Diagnosis: Right Breast Mass, Tru Cut Biopsy:

Invasive Duct Carcinoma (NST), Grade II
(ER 4/8), (PR 3/8), (He2 Score 1+), (Ki67 25%).

Signature:

Dr.Nermine Nabil.M.D

Dr.Mona Mohamed.M.D

Pathology Lab. Manager

Nermine Nabil, M.D

Patient Name : الأستاذ / كوثر عادل عبد العزيز	Request No. : 13005784
Patient No. : 28409200108303	Request Date : 09/04/2026
Path No. : 947/26	Verification Date : 20/04/2026
Sex : Female	Financial Category : نفقة الدولة - جومستاف رومسي
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer (Post-NACT)
Nature of specimen: Right modified radical mastectomy.

Gross Examination:

- Right modified radical mastectomy specimen measured 30x20 cm, with covering skin ellipse measured 25x10 cm. Nipple & areola looked unremarkable grossly.
- Serialling showed a tumor bed site measured 5x4x2 cm, located 2.0 cm away from the deep margin.
- Axilla: Axillary fat measured 8x4x1 cm, entangling 10 lymph nodes, the largest measured 2x1x1 cm.
- 11 Blocks:
- 1:5 T.B.S., 6- Nipple & areola 7- Deep margin., 8:11- ALNs.
- Gross was done by: Dr. Heba El Shafeey.

Microscopic examination:

- * Histological type: No residual viable tumor tissue detected.
- * Ductal carcinoma in situ: <1% Minimal high grade DCIS.
- * Periductal invasion: Absent.
- * Angiolymphatic invasion: Present.
- * Microcalcification: Present.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Positive metastatic deposits of high grade carcinoma grade 3 (Metaplastic carcinoma with squamous cell carcinoma) involving one out of sixteen lymph nodes (1/16) (The largest nodal mets measuring 0.5 cm, with extra-nodal extension).
- * Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Verified by :
Dr. Heba Hasan

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Page 1 of 2

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Patient Name : الأستاذ / كوشر عادل عبد العزيز	Request No. : 13005784
Patient No. : 28409200108303	Request Date : 09/04/2026
Path No. : 947/26	Verification Date : 20/04/2026
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Right breast cancer (Post-NACT), Right modified radical mastectomy:

- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: <1% Minimal high grade DCIS.
- Nipple & areola: Free.
- Deep margin: Free.
- Axillary lymph nodes: Positive metastatic deposits of high grade carcinoma grade 3 (Metaplastic carcinoma with squamous cell carcinoma) involving one out of sixteen lymph nodes (1/16) (The largest nodal mets measuring 0.5 cm, with extra-nodal extension).
- * Histologic response to therapy: Pathologic partial response to therapy.
- Stage: ypT0, ypN1a (slr).
- RCB: 1315, RCB class I

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :
Dr. Heba Hasan

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CV ٢٥

ECHO REPORT

NAME: كوجر عادل بن الزبير DATE: 31/3/2026
ID NUMBER: 2262 / 25 SEX: ♀ AGE: 41

F	66.1	55-75%	PWT	0.9	0.6-1.1cm
VEDD	4.3	3.5-5.8cm	AO.R	2.8	3-3.7cm
VESD		2-4cm	LA	3.3	2-4cm
S		25-45%	RV		0.7-2.3com(supine)
WT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions ———— amcl
LV Wall Thickness ———— amcl
LV Diastolic Function DD grade I
SWMA ———— 0 SW TA
LA and Ao Root Dimensions ———— amcl
Mitral Valve ———— amcl
Aortic Valve ———— amcl
Tricuspid Valve ———— Inval TR
Pulmonary Valve ———— amcl
Right Side and Systolic Function ———— amcl
Intracardiac Masses and Effusion ————

DR.
J. J. J.



LABS

Collection Date: 29/12/2025
Review Date: 03/02/2026

Patient Name: الاستاذة كوثر عدل عبدالعزيز عبدالحمد
Patient No.: 239-7419
Accession No.: 239251218628
Age: 41Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Clinical Information

Kawthar Adel is a Cancer patient, tested for germline mutations in BRCA 1,2 for the possible applicability for targeted therapy.

Test Information

Extracted DNA using QIAamp DNA Mini kit from the patient's blood sample had successfully passed appropriate libraries according to the AmoyDx BRCA Pro Panel and sequenced on the MiSeq platform (Illumina). The minAmp (minimum depth) was not less than X300.

- The coding regions of BRCA1 & BRCA2 genes have been investigated using NGS technology.
- BRCA1 & BRCA2 genes have been implicated in hereditary breast, ovarian, prostate and pancreatic cancer predisposition syndrome.
- For patients with BRCA1/2 mutations (actionable pathogenic or Likely pathogenic Variants). The PARP inhibitors show clear benefit (Gradishar et al., 2021).
- All identified variants are evaluated with respect to their pathogenicity and causality, and were categorized into classes (1-5) as recommended by ACMG guidelines, where Class (1) Pathogenic, Class (2) Likely pathogenic, Class (3) Variant of Uncertain Significance (VUS), Class (4) Likely benign, Class (5) Benign (Richards S, et al., 2015).
- Our variant classification pipeline included variant interpreter by Illumina, Clinvar and VarSome (Kopanos C, et al., 2019).
- Only pathogenic and likely pathogenic variants with actionable therapeutic application are reported.
- The test results were reviewed, interpreted and reported by our scientific and medical experts.
- The classification of variants can change over the time.

Test result

No Actionable Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

No large rearrangement or Copy Number Variations (CNV) in BRCA1,2 genes.

Interpretation

The patient is not harboring Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

The patient is unlikely to benefit of PARPi.

Recommendations

MLPA analysis of BRCA1 and BRCA2 genes is recommended to rule out the presence of large deletions/duplications or copy number variations.

We recommend Follow up with the referring Physician.

Please feel free to contact NGS team at Alfa laboratories (NGS@alfalabs.com).

R. Branch: قسطون

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AMG

ALFA LABORATORIES

16191



LABS

Collection Date: 29/12/2025
Review Date: 03/02/2026

Patient Name: الاستاذة كوثر عدل عبدالعزيز عبدالحمد
Patient No.: 239-7419
Accession No.: 239251218628
Age: 41Y

NEXT GENERATION SEQUENCING (NGS)

GENE-EXPRESSION-PCR

[[DOCTYPE:OTHER_GENE_EXPRESSION_PCR]]

AMG

A MEMBER OF ALFA MEDICAL GROUP

معمل الفا
16191

L A B S

Collection Date: 29/12/2025
Review Date: 03/02/2026

Patient Name: الاستاذة كثر مائل عبدالعزيز عبدالحديد
Patient No.: 239-7419
Accession No.: 230251218528
Age: 41Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Test limitations

1. Intronic variants/variants in untranslated region, promoters and intronic variants are not assessed using this method.
2. Variants not detected by the assay that was performed may impact the phenotype.
3. This test is using reference databases obtained from occidental communities.
4. Cases where the genetic variant is not identified may be due to limitations in current medical knowledge or testing technology.
5. Genetic tests are largely dependent on the clinical history, medical findings and family pedigree, and therefore clinical accurate interpretation of test results may require knowing the true biological relationships in a family. Misinterpretation of results may occur if the information provided is inaccurate or incomplete.
6. The interpretation of test results is done in accordance with the clinical history, family history and other medical and laboratory data.
7. Inaccurate results in genetic testing are very rare as these tests are highly specific and accurate.

References:

1. Garrett et al., BRCA1/BRCA2 :COSMIC-UK Gene-Specific Guidance 2024 (<https://www.cancereconomics.org/cosmic-uk-guidance>).
2. Grodsher WJ, Moran MS, Abraham J, et al. NCCN Guidelines® Insights: Breast Cancer, Version 4.2021. J Natl Compr Canc Netw. 2021;19(5):484-493. Published 2021 May 1. doi:10.6004/jnccn.2021.0023
3. Kopylov C, Tsickas V, Kouris A, et al. VarSome: the human genomic variant search engine. Bioinformatics. 2019;35(11):1978-1980. doi:10.1093/bioinformatics/bty697
4. Richards S, Aziz N, Bale S, et al. Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology. Genet Med. 2015;17(5):405-424. doi:10.1038/gim.2015.30

Authorized and Reviewed by:

Dr. Samar Saad

Consultant, MD

Prof. Sehar AbdWaty

Head of the NGS Unit

Reviewed By:



PL Branch: جدة

Page 2 of 2

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معمل الفا
16191

جمهورية مصر العربية
بطاقة تحقيق الشخصية



كوش
عادل عبدالعزيز عبد الحميد
٤٠ ش الاحرار - جنال عبدالناصر - شنتل
مركز اوسيم - الجيزة

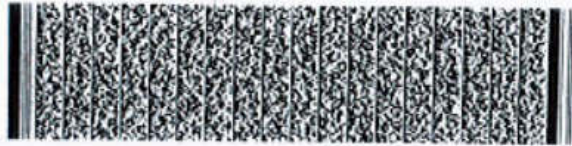
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ربة منزل

أنثى مسلمة
محمد فتحي خميس

البطاقة سارية حتى ٢٠٣٠/١٢/٣١



DMS
دوبلر على الرفية
مقطوعة على الصبر
سوار على البورتناكات



مجلس الوزراء
مملكة البحرين

مؤثر
علاء عبدالعزیز عبدالحمید

٤٠ ش. الاحد از جمال عبدالناصر - بشتين
مركز او ستم - الجزيرة

KJ1747475

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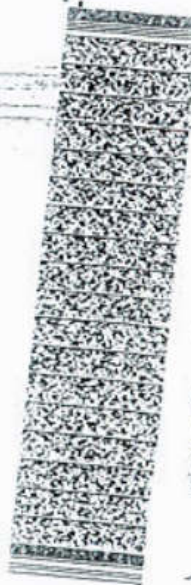
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مؤثر

مملكة

مجلس فني

البحرين مملكة البحرين



Medical No. : 4146
Patient's Name: Kawthar Adel Abdulaziz Abdulaziz
Date: 2026-09-21
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, routine follow up.

Findings:

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid lesions with few sub-centimetric simple cysts
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:-

***Left breast (BIRADS 2): Mild fibrocystic changes.
Clear right mastectomy bed and axilla.***

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	